09 — Phase 2 Decision Guide: What Should We Build?

Phase 1 is paper-first so the project can start without depending on any system. Phase 2 decides whether a tool would improve reliability, and which one.

Open questions (must answer first)

#	Question	Why it matters	Answer
1	**What does the clinic use to document today?** Paper, EHR (which one?), spreadsheet?	Decides whether we build inside an EHR or standalone	*Unknown — find out*
2	Does the clinic have tablets/ phones and reliable cellular data on the van?	Offline capability may be required	
3	Is the clinic an FQHC / Health Care for the Homeless grantee reporting UDS?	May require PRAPARE instead of AHC-HRSN	
4	Does the county or region use a closed-loop referral platform (e.g., Unite Us, findhelp)?	Integrate with it rather than build	
5	Who will own and maintain any tool after the student leaves?	Avoid building something unsupported	
6	What is the project timeline?	Limits how much can be built and tested	*Not decided*
7	Where may data be stored (HIPAA-compliant, clinic-controlled)?	Rules out many free tools	
8	Will patients self-administer on a tablet, or will staff read the questions?	UX design, literacy	
9	Is there a confirmed clinic partner and an approved route for getting data out of the clinic?	No real data or testing without one; see 10	*Not yet*

Build options

Option	Effort	Pros	Cons	Fits when
A. Stay on paper + spreadsheet (this repo's templates + `analyze.py`)	Very low	Zero cost; works offline; no IT approval	Transcription errors; manual follow-up tracking	No devices or IT support; short timeline
B. EHR-native form/ flowsheet	Low–medium (EHR analyst)	Data in the chart; reportable; sustainable	Needs vendor/IT support; less flexible	Clinic has an EHR with a configurable template

Option	Effort	Pros	Cons	Fits when
C. REDCap or similar secure survey tool	Low–medium	HIPAA-capable via institution; offline mobile app; built-in exports	Separate from the chart; institution must host	Academic partner with REDCap; no EHR flexibility
D. Existing referral platform	Low (config)	Closed-loop referrals built in	Licensing cost; partner adoption varies	Region already uses one
E. Custom app (offline PWA: screening + referral log + dashboard)	High	Tailored workflow and dashboard	Security, hosting, HIPAA, and maintenance burden	Only if A–D are not feasible and there is a long-term owner

Current recommendation: start with **A**. Move to **C** (REDCap) or **B** (EHR) when Question 1 is answered. Option E is the last resort. If a custom tool is built, a **QI dashboard** that reads the coded CSV logs (no direct identifiers) and runs on clinic-controlled storage is the safest and most useful thing to build first.

What Phase 1 already provides

- A data schema (data dictionary) that every option can export to.
- Analysis code that produces the run charts from any option's CSV export.
- A paper form that can be converted directly into REDCap or EHR fields.